



Mental Disorder and Mysticism in the Late Medieval World

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During the later Middle Ages, a number of religiously oriented people behaved in ways that we would consider unusual, yet it was unusual for them to be regarded as mentally disordered. This article reviews late medieval thinking and practice with regard to mental disorder and also with regard to the discernment of spirits, that is, how it could be decided whether an experience or impulse to do something was the consequence of God or a good spirit, an evil spirit, or some purely human cause. Many of the criteria for discerning a good spirit were behavioral, for example, consistently showing humility and discretion, and were clearly distinct from those displayed in mental disorder. A comparison of the criteria for mental disorder with those used to discern spirits shows how the distinction between mental disorder and religious experience could have been made and why confusion of the two seems to have been rare.

Keywords: discernment of spirits, mental disorder, Middle Ages, mysticism

To the modern eye, the behavior of many religiously oriented people in the European Middle Ages seems odd, perhaps evidence of psychotic thinking. Many people were religiously motivated; some practiced self-discipline to the point of self-harm; some claimed visions or even prophetic power. Yet although their contemporaries often regarded reports of religious experiences with skepticism, for the most part they did not regard them as evidence of mental disorder. In this article I examine how, in the later Middle Ages, people considered mental disorder and mystical experience and the different criteria they used to demarcate them. If we regard the two sets of criteria side by side, we can see why they seem to have usually found the experiences easy to distinguish.

By the 13th century, there was a well-established tradition of believing that people could suffer from mental disorders of different kinds. This tradition, the diagnoses arrived at, and the consequent treatments are well known to modern historians and are summarized in the following section. Although, occasionally, such disorders might be attributed to diabolic causes such as demonic possession, for the most part they were attributed to environmental or biological causes. Such diagnoses were known not only to medieval physicians but also, for example, to medieval theologians.

Also in the later Middle Ages, most people, and certainly most people in official positions in the church, believed in a Christian God. Moreover, many believed that some people, of whom the best known is, and probably was, St. Francis of Assisi (1182/3–1226), had real experiences in which they were mystically united with this God. Such experiences were often sought after, both in monastic communities and outside them, and

This article was published Online First February 7, 2019.

I am grateful for helpful and constructive feedback from Martin Dorahy and Julia Rucklidge. I am also grateful for opportunities to use the library of the Warburg Institute in London.

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it would probably not have been very unusual to encounter someone who reported an important experience that they thought might be of this kind. Relatively few of these reports were believed, but there was general agreement that some might be genuine. Devising reasonable criteria for the genuineness of these experiences was an ongoing concern of the medieval church that was called the *discernment of spirits*. The present article analyzes the different criteria that were used and shows that there was considerable agreement about them across different writers. It turns out that most criteria were based on the character of the mystic and could be assessed from observable behavior.

The article points out that the criteria for discerning a “good” spirit were incompatible with those for discerning mental disorder. For highly regarded mystics, St. Francis of Assisi, for example, little confusion was likely. However, the careers of other mystics and would-be mystics indicate that they, too, were rarely suspected of mental disorder, even though it was widely recognized that ascetic activities such as depriving oneself of sleep or eating poorly or beating oneself could lead to mental disorder. Huot (2003) commented that a concern of medieval hagiography—that is, writing about saints or possible saints—was the “crucial but fraught distinction between the mad person and the saint” (Huot, 2003, p. 28). The present article provides an account of how people in the later Middle Ages could have made this distinction. Essentially, the distinction was based mostly on the perceived character of the person reporting the experiences, and this character was evident from that person’s behavior.

Views of Mental Disorder in the Later Middle Ages

There is now a wealth of writing and evidence concerning the views of mental disorder that were prevalent from the 13th century to the end of the 15th century. This includes general overviews of approaches to mental disorder (e.g., Clarke, 1975; Craig, 2014; Kemp, 1990), accounts of particular models of mental disorder used by medieval practitioners (e.g., Jackson, 1986; Lennox, 1941; Schipperges, 1967), and exploration of the legal and institutional aspects of mental disorder (e.g., Neugebauer, 1978, 1979; Turner, 2010b, 2013). There are also accounts by onlookers in the Middle Ages who suspected or believed that others were mentally disordered. Brief accounts of this kind can be found in a range of sources, including religious shrines (e.g., Clarke, 1975), canonization hearings (Kuuliala, 2016), legal records (e.g., Neugebauer, 1979; Turner, 2010a, 2013) and administrative records of clergy (King, 2010). The poet Thomas Hoccleve (ca 1368–ca 1430) complained of his own temporary mental disorder in one of his poems (Hoccleve, 1420/1999). For some individuals, for example, the late 15th-century painter Hugo van der Goes (Stechow, 1966) and the 14th-century geographer Opicinus de Canistris (Heinrichs, 2003), there are reasonably detailed accounts written by observers. There are also detailed accounts for some medieval rulers, the detail probably arising from the widespread consequences of their mental disorders. Thus, we know a good deal about the disorders suffered by Henry VI of England (1421–1471; e.g., Green, 1993; Rushton, 2010), Charles VI of France (1368–1422; e.g., Famiglietti, 1986; Green, 1993), and Duarte of Portugal (1391–1438; McCleery, 2009), as well as more minor rulers (e.g., Midelfort, 1994). Given the extent and accessibility of this previous work, I summarize only some salient points here.

In our own day, the term *mental disorder* is a recognized umbrella term for a range of complaints. It forms part of the title of the *Diagnostic and Statistical Manual of Mental Disorders* (5th ed.; American Psychiatric Association, 2013). The anachronistic use of *mental disorder* in this article is a consequence of the lack of a corresponding medieval term (at least in English or Latin). Instead there was a variety of different terms. For example, the Latin terms *non compos mentis*, *non sane mentis*, *furiosus*, *insanus*, *mania*, and *stultus* were used, and although their meanings are sometimes distinguishable, different writers used the terms differently. To sum up, medieval terminology is vague (for

more detailed discussion, see [Turner, 2013](#), pp. 17–24, 78–81, 233–238). Incidentally, this vagueness extended to other sets of terms. For example, medieval terms for cognitive functions of the brain and different physical disabilities also varied greatly from writer to writer ([Kemp, 1996](#), pp. 51–52; [Kuuliala, 2016](#), pp. 32–48).

Despite problems with the terms, it is possible to make general statements about the medieval approach to mental disorder. In the first place, the ideas were linked to medieval cognitive psychology. Very briefly, medieval theory viewed human cognitive function as occurring partly in the immaterial and immortal intellect and partly in the material and mortal brain, and specifically in three ventricles of the brain (e.g., [Harvey, 1975](#); [Kemp, 1996](#); [Kemp & Fletcher, 1993](#)). Humans possessed an immaterial intellect; both animals and humans possessed ventricles. In the more sophisticated versions of this theory, especially that of Avicenna (980–1037), recall of associations or visual imagery resulted from the interplay of the immaterial intellect with the memory stores in the front and rear ventricles ([Avicenna, ca 1020/1968](#), [ca 1020/1972](#); [Kemp, 1996](#), Chapters 4 and 6). Cognitive functions that relied on the proper functioning of the brain were thus affected by physiological conditions, and mental disorder was generally held to result from a disorder of the brain or other part of the body rather than damage to the intellect itself; nonetheless, intellectual functioning would be impaired because the intellect could not make proper use of the brain.

This theory connected well to the practical determination of whether someone was mentally disordered. Tests for disorder often involved simple tests of basic cognitive functioning, for example, the ability to count ([Neugebauer, 1978, 1979](#); [Turner, 2013](#)) or to have a reasonably functioning memory ([Turner, 2010a, 2010b, 2013](#)). The abilities to function in society and perform one's job adequately were important ([King, 2010](#); [Neugebauer, 1978](#)), as was the display of appropriate emotion ([Turner, 2010b, 2013](#)). The frequent inability of King Henry VI of England to make useful decisions or even, on occasion, to speak at all was an obvious symptom of his disability ([Rushton, 2010](#)).

Medieval medical accounts of mental disorders tended to stress physiological causes. Medieval physicians identified a number of different disorders, including melancholy, mania, phrenesis (or frenzy), lethargia, and perhaps epilepsy. Probably the best known and most written about of these was melancholy. The word derives from the Greek for black bile, and two accounts of it that were very influential in the Middle Ages, those of [Galen \(192/1976\)](#) and [Constantine of Africa \(ca 1070/1977\)](#), saw the disorder as caused by the vapors of black bile ascending to the brain. Both also characterized it as giving rise to great fear and sorrow without an appropriate cause. Sufferers might have grotesque delusions. For example, [Galen \(192/1976, p. 93\)](#) reported one patient who feared that he would be crushed because he thought he was a snail; Constantine of Africa reported a patient who believed he had no head (p. 120). Both authors pointed to different types of melancholy that produced different types of symptoms and might require somewhat different treatment. Constantine of Africa remarked, too, that melancholy could give rise to epilepsy, and vice versa, and that many lepers became melancholic (p. 131). This picture was complicated, even contradicted, by a tradition of identifying mental disorders by the ventricle of the brain that was supposed to be affected: Mania was produced by a disorder of the front ventricle, melancholy by a disorder of the middle one, and lethargia of the rear one (e.g., [Bartholomaeus Anglicus, 1397/1975](#), pp. 349–351).

If a mental disorder was believed to arise from a humoral disorder, and melancholy was not the only example (e.g., phrenitis or frenzy was often associated with disordered yellow bile), then it was logical to attempt a cure by restoring the humoral balance. One way to do this was to change what people ate and drank, and dietary change was often recommended (e.g., [Constantine of Africa, 1070/1977](#), Book 2).

There were few institutions specifically dedicated to caring for the mentally disordered in the Middle Ages. Some European hospitals did house them, not always in particularly good conditions ([Knefelkamp, 1989](#), pp. 204–210; [Walker & McCabe, 1973](#), p. 2), but

hospitals were likely to prefer treating patients with acute (and curable) rather than chronic disorders, and most mental disorders were regarded as chronic.¹ When resources were available, sufferers might be cared for privately (King, 2010; Neugebauer, 1978; Turner, 2013). This was particularly true for mentally disordered kings (Green, 1993), but accounts of the life of Charles VI indicate that, as the prospects of a real cure grew ever fainter with time, the quality of even royal care deteriorated (Famiglietti, 1986).

Nonphysiological causes of mental disorder—either sole or interacting—were also admitted. Constantine of Africa (1070/1977, pp. 103–104) thought that ascetics were prone to melancholy and that it could arise from financial disaster or the loss of a child. He also identified lovesickness as a disorder, that might be treated by a variety of pleasant remedies: sex, wine, and conversation with interesting and good-looking friends (Wack, 1990, pp. 186–193). It is hard to know how many people suffered from lovesickness in medieval Europe, but it was a theme in medieval literature (Huot, 2003; Kolve, 1997). For example, Chrétien de Troyes' Yvain went mad after he was rejected by his wife. He lost his identity as a knight and ran wild in a forest until he recovered his sanity with the aid of heroic acts (Chrétien de Troyes, 1180/1975). Hoccleve's disorder appeared to echo this tradition: He claimed to have lost his memory and the ability to speak for some months (Hoccleve, 1420/1999). In *The Knight's Tale*, Geoffrey Chaucer (ca 1390/1966) described Arcite as suffering from lovesickness that resembled mania produced by a melancholic humor in a ventricle of the brain.

More prosaically, John of Mirfield (ca 1390/1969, pp. 49–51) noted in the *Breviarium Bartholomei* [Summary for Bartholomew] that people who had suffered blows to the head sometimes showed intellectual impairment or disordered dreams. English Courts of Chancery seem to have mostly identified everyday causes of mental disability, for example, age, a blow to the head, or fright (Neugebauer, 1979).

As *The Knight's Tale* indicates, medical lore about mental disorder was often known to educated people in the later Middle Ages. In particular, theologians knew about it. Jean Gerson (1363–1429), an academic theologian who became Chancellor of the University of Paris and whose work on discerning spirits is referred to in the next section, was a good example of this (Hirvonen, 2018). Perhaps, more surprisingly, the knowledge was also demonstrated and apparently accepted by the authors of the infamous book on witch hunting, *Malleus Maleficarum* [The Hammer of the Witches] (Kramer & Sprenger, 1487/1986, pp. 264–265, 297).

As is also considered in the next section, many later medieval theologians (and lay people) believed that devils had the power to tempt, deceive, and obsess people. Indeed, on occasion they were believed to possess them (Kemp & Williams, 1987). These beliefs were not held in opposition to the medical views but often integrated with them. For example, the cognitive powers believed to be present in the cerebral ventricles could be disturbed or infiltrated by devils (e.g., Hirvonen, 2018; Kramer & Sprenger, 1487/1986, pp. 264–265). Thus, diabolic influence could mimic or aggravate a naturally caused disorder.

Even from this brief summary of medieval thinking about mental disorder, it should be clear that there were a number of possible diagnoses and hypothesized causal agents of different kinds. Given this diversity of possible causes, it is unsurprising to find in medieval accounts of disordered individuals that, often, no definite diagnosis was reached. So, for example, there does not seem to have been any agreed diagnosis or cause for the disorders suffered by van der Goes, Opicinus, Henry VI, Charles VI, or Duarte, and in at least two cases (van der Goes and Duarte), it remained unclear whether the cause was physiological or diabolic. In the case of Charles VI, uncertainty did not arise for want of

¹ Wards specifically for the mentally disordered seem to have been more common in the Islamic world, as was the practice of confining dangerously mad people in them (Ragab, 2015, pp. 193–199).

trying. The shambles to which his disorder reduced French government led his courtiers to resort to a variety of different practitioners who employed a variety of different treatments based on a variety of different hypotheses as to the cause of the problem. However, in the end, his mental disorder, and presumably that of many less intensively examined sufferers, went undiagnosed and uncured.

The Discernment of Spirits

The discernment of spirits, which concerns whether an apparent vision or inspiration is of divine, angelic, diabolic, or purely human origin, was an important issue in the later Middle Ages. The basic issue was simple. An individual might have a vision or dream or insight or impulse to do something that he or she felt could be some kind of personal attention from God, a saint, or an angel. The question then arose: Was it really?

Reports of such experiences often met with skepticism. Doubts about people who claimed or suspected that they might have had personal contact with God or another heavenly figure have a long history, and there are skeptical references in both the Old and New Testaments (e.g., Deut. 18:21; Ezek. 22:28; Matt. 7:15; 2 Cor. 11:4–15). One of the more cited of the biblical references (1 John 4:1; Revised Good News Edition) reads, “Do not believe all who claim to have the Spirit, but test them to find out if the spirit they have comes from God.” This passage and advice were taken seriously in the Middle Ages.

The tradition of believing that some visitations were really from God or other heavenly source—good spirits—whereas other visitations were deceptive works of a devil or simply the product of some human physiological process continued throughout the Middle Ages. Indeed, it is followed by many Christians in the present day (e.g., Lindsey, 1972). However, a number of features of the way that Christianity developed in the later Middle Ages ensured that it was a particular concern of that period. Note, first, that in medieval Christianity, unlike in Islam or Judaism, there was a widespread belief that personal revelations from God remained possible (Anderson, 2011, p. 2). Second, from the 13th century onward, and in part following the examples of Mary of Oignes (1176–1213) and St. Francis of Assisi, there was an upsurge of interest in having personal spiritual experiences that extended to lay people outside of the monastic environment (McGinn, 1998, Chapter 1; Trinkhaus & Oberman, 1974). Indeed, many people became highly motivated to achieve unity with God (McGinn, 2001; Milhoven, 1993). Third, following the discovery of the Catharan heresy and the Albigensian crusade (1209–1229), the Church became increasingly concerned that personal religious experience did not lead to the spread of heresy. Fourth, the papal move to Avignon (1309–1376) and the Papal Schism (1378–1417) undermined the central authority of the Church (Anderson, 2011; Caciola & Sluhovsky, 2012).

Overall, in this period, a few of the claims (or sometimes suspicions) of having been personally visited by a good spirit were believed, and most were not. Outsiders judging the accuracy of the claims did not usually believe that false claimants were deliberately lying. Rather, they often believed the claimants to be mistaken as to the source of the vision, and that it was either the work of the devil or the body itself rather than a divine or angelic influence. A number of people wrote guidelines for discerning the nature of the spirit, some intended for the person experiencing the event, some for outsiders, particularly confessors, and some for both. Later medieval theologians who did this included Peter of John Olivi (1248–1298), Henry of Friemar (ca 1285–1354), Henry of Langenstein (ca 1325–1397), and Jean Gerson.

These guidelines often referred to the Bible, including the examples cited above, and to earlier Christian writing. For example, St. Augustine (354–430), in his *Literal Meaning of Genesis*, provided a physiological and psychological account that distinguished corporeal, spiritual, and intellectual vision. Both good and evil spirits could produce corporeal and spiritual visions, but intellectual vision was not deceived (Augustine, ca 426/1982,

Book 12). The theologian John Cassian (360–435) also produced an early treatise on discerning spirits, which emphasized the cunning and power of the devil and the importance of discretion in deciding the true cause of one's own experiences. An outsider might look for evidence of charitable behavior and calmness and tranquility as indications of a good spirit (Cassian, ca 410/2018). Saint Bernard of Clairvaux (1090–1153), who played an important role in the founding of the Cistercian order of monks, pointed out that good and evil spirits could be distinguished by their effects. For example, the spirit of evil produced anger, impatience, envy, and bitterness of the soul. (Bernard of Clairvaux, ca 1130/1667, p. 2379).

The number of distinct spirit types varied with the writer. For example, Bernard of Clairvaux (ca 1130/1667, pp. 2377–2381) distinguished seven, whereas Henry of Langenstein (1388/1977, pp. 52–56) distinguished 16. However, the broad categorization into human, diabolic, and heavenly spirits seems to have been widespread. Also widespread was a belief that discerning the nature of the spirit was difficult and that mistakes could be expected. Gerson (1402/1998, p. 335) commented that “there is for human beings no general rule or method that can be given always and infallibly to distinguish between revelations that are true and those that are false or deceptive.”

Given that no one rule seemed to be adequate for discerning spirits, most writers mentioned a number of different pointers, guidelines, or tests. Often, these tests were of personal attributes. The pointers given varied somewhat from writer to writer, but there was also considerable overlap. Pointers given by at least two of the writers mentioned so far in this section include humility, discretion, patience, truth, charity and good works, and calmness and tranquility, each of which are discussed in turn.

Regarding *humility* (Bernard of Clairvaux, 1130/1667, p. 2380; Gerson, 1402/1998, pp. 337–342; Henry of Friemar, 1315/1977, pp. 156–159; Henry of Langenstein, 1388/1977, p. 72; Peter of John Olivi, ca 1280/2012, pp. 28–29), the possible recipient of divine favor was supposed to be humble in everyday life and was not supposed to believe that he or she was in any way deserving of the favor. Gerson wrote, “if he thinks himself worthy of [it] and delights in boastful telling of such matter, then know that he deserves to be fooled” (Gerson, 1402/1998, p. 338). Indeed, the true recipient was often supposed to disbelieve that the revelation had come from God, at least at first, because of a conviction of his or her own unworthiness.

Regarding *discretion* (Bernard of Clairvaux, ca 1130/1667, p. 2380; Cassian, ca 410/2018; Gerson, 1402/1998, pp. 337–342; Henry of Langenstein, 1388/1977, p. 72; Peter of John Olivi, ca 1280/2012, pp. 28–29), would-be visionaries were expected to employ reason, common sense, and some skepticism. As one aspect of discretion, Henry of Langenstein (1388/1977, pp. 70, 76, 114–118) and Gerson (1402/1998, pp. 343–347) recommended against excessive fasting and excessive religious zeal. Another important aspect of discretion was asking advice about one's experience from others, especially a confessor (e.g., Peter of John Olivi, ca 1280/2012, p. 32). Conversely, a refusal to accept advice from others was an indication of either pride or an evil spirit. Note that humbler people would be more likely to ask and follow advice, so there was some overlap of humility and discretion.²

Regarding *patience*, both Gerson (1402/1998, pp. 340, 348) and Peter of John Olivi (ca 1280/2012, pp. 30–31) thought it important not to make a hasty decision about a revelation but instead to delay taking action. Gerson quotes Moses's reluctance to act as a prophet (Exod. 4:13). There are obvious relationships of patience to both humility and discretion. The humble person would delay belief because he or she felt unworthy. A

² Late medieval writers on the discernment of spirits appeared to take it for granted that it is rational to believe in God and follow His prescriptions. There was a medieval tradition of thinking that those who denied God were foolish (e.g., Kolve, 1997).

delay could also come about because a rational person would seriously question the reality of the experience.

Considering *truth* (Gerson, 1402/1998, pp. 348–350; Peter of John Olivi, ca 1280/2012, pp. 29–32), some revelations contained prophecies; if these did not come true, then they could not have come from God. However, the reverse did not hold, because coincidence or the devil could also have produced accurate prophecies (e.g., Augustine, 426/1982, pp. 201–202). A more immediate test of the truth of a revelation was whether it was in accord with the Bible and the examples of the saints. If the truth or doctrine contained in an apparent revelation was to be judged by a confessor, for example, discretion on his part would be required.

Regarding *charity and good works* (Bernard of Clairvaux, ca 1130/1667, p. 2380; Cassian, ca 410/2018; Gerson, 1402/1998, pp. 355–357; Henry of Langenstein, 1388/1977, pp. 92–106; Peter of John Olivi, ca 1280/2012, p. 31), although the Latin word *caritas* is often translated as *charity*, it had a wide medieval meaning to encompass feelings and acts of loving kindness toward other people and God (see, e.g., Kemp, 2018). People who acted with such charity were likely to have been visited by a good spirit. Conversely, people who acted enviously, who were embittered, and who got into conflicts with others were not (e.g., Bernard of Clairvaux, ca. 1130/1667; Cassian ca 410/2018; Henry of Langenstein, 1388/1977, p. 74).

Concerning *calmness and tranquility* (Bernard of Clairvaux, ca 1130/1667, p. 2381; Cassian, ca 410/2018; Gerson, 1402/1998, p. 360; Henry of Friemar, 1315/1977, pp. 154–162; Henry of Langenstein, 1388/1977, p. 64), a true religious experience was supposed to lead to calmness and comfort, even though it might initially be disturbing (as Mary was troubled by the Annunciation; Luke 1:29). Henry of Friemar (1315/1977, p. 158) wrote of mildness opening the heart and mind with sweetness, and this happening as a consequence of humility. By contrast, a visit from an evil spirit might initially be pleasant, but the experience would later prove bitter.

Two signs were regarded as unreliable. Personal austerity, in which someone mortified the body with fasting, sleeplessness, or painful clothing, was not necessarily taken as a sign of good spiritual influence, and late medieval writers warned against excessive austerity (e.g., Gerson, 1402/1998, pp. 343–346; Henry of Langenstein, 1388/1977, p. 698). The apparent ability to perform miracles was not taken as decisive either. Gerson (1402/1998, p. 342) pointed out that evil people can perform miracles. In line with the importance of humility, it was quite wrong for people to expect or even ask for a miracle (Henry of Langenstein, 1388/1977, pp. 76–80).

The emphasis given to the different pointers varied from writer to writer. For example, Gerson (1402/1998) put greatest stress on humility, Henry of Langenstein (1388/1977) on discretion. However, given the obvious overlap and interplay between the pointers, it is hard to regard these differences as important. Indeed, the writers themselves often acknowledged the interplay between the different attributes. As a single example, Henry of Friemar (1315/1977, p. 158) claimed that calmness and sweetness result from the humble person inviting God to enter his or her soul like a spouse inviting one to the bedchamber.³

Spirits, Psychology, and Mental Disorder

The pointers of humility, discretion, patience, charity, and calmness, and tranquility can be thought of as characteristics or attributes of the individual, and they could be assessed by observers from how the individual behaves. Essentially, then, the criteria for deciding

³ “Sed ipsa humiltas Deo tam suaviter redolens ipsam animam quasi ad sponsi cubiculum introducit et eam divinis amplexibus coniungit.”

whether someone had received a genuine revelation or other visitation from a good spirit were based on behavior. Moreover, the criteria were usually referenced to the long-term behavior patterns shown by the would-be mystic rather than just how he or she reacted to the experience. For example, humility would be displayed not just in people declaring themselves unworthy of a particular religious experience but in how they interacted with those around them on a day-to-day basis.

It is also evident from even a cursory comparison of these personal characteristics with the symptoms of melancholia or mania that they are very different. Consequently, confusion was unlikely. People who were not proud and boastful but calm and cheerful, showed concern for the well-being of others, regarded their own religious experiences or those of others with skepticism, and were happy to quietly seek advice from others would not meet the medieval criteria for mental disorder, no matter what religious experiences they had.

A wide separation between the nature of true revelation and that of mental disorder was indicated by theory as well as behavioral symptoms. The medieval model of mental disorder stressed the attenuation of the power of reason, but true visitations from good spirits were thought to be essentially rational. [Henry of Friemar \(1315/1977, p. 170\)](#) wrote that the angelic instinct should move people to what is good and useful in itself.⁴ In line with his belief in the importance of discretion, [Henry of Langenstein \(1388/1977, p. 68\)](#) pointed out that for someone to proceed foolishly, irrationally, and indiscreetly was odious to God, because this was contrary to the nature of man to whom God gave reason.⁵ Another aspect of this stress on reason was the advice that other people should be called on to judge the truth of revelations because they were likely to be better judges than the individual concerned ([Gerson, 1402/1998, p. 353](#)).

An example of how this worked out in practice is given in the medieval biographies of St. Francis of Assisi. St. Francis often behaved unusually, as when he preached to birds, and indeed very early on in his religious career, he was briefly thought to have become insane ([Thomas of Celano, 1228/1973a, p. 238](#)). However, in later years, he was both widely respected in medieval society and widely credited with having personal experience of God. There are many recorded instances of his humility (e.g., p. 273); patience, discretion, and common sense (e.g., pp. 283, 287); charity; and peace and cheerfulness of mind. There are also many examples given of his heeding advice from church officials, even when this went contrary to his own inclinations. Thus, his biographers ([Thomas of Celano 1228/1973a, 1240/1973b; St. Bonaventure, 1263/1973](#)) portrayed—or, perhaps, were careful to portray—St. Francis as exhibiting the characteristics of a man who might indeed receive true revelations.⁶

Thus, in late medieval Europe, a clear distinction could be made between those suffering from mental disorder and those who were credited with personal experiences of God. However, very few medieval people who might claim revelation were likely to meet the behavioral criteria that St. Francis of Assisi could. One might expect that here there would be greater scope for regarding the more suspect claimants as mentally disordered, and there is some evidence for this.

As remarked earlier, theological writers often had a good understanding of contemporary theories of mental disorder, and they did recognize that some of those claiming revelation might be mentally disordered. [Gerson \(1402/1998, p. 338\)](#) remarked that some “ascribe to revelation even superstitious, vain and illusory deeds and dreams of mad people, and they believe in the portents made up by those who are mentally ill or in

⁴ “Instinctus angelicus semper movet hominem ad illa, quae sunt bona et utilia in se ipsis.”

⁵ “Quia hominem fatue, irrationabiliter et indiscrete procedere est deo valde odibile; cum hoc sit maxime repugnans conditioni hominis, cui deus dedit rationem.”

⁶ Many medieval biographers would have been well aware of the criteria for identifying good spirits, and hence of the importance of relating anecdotes showing their subjects had the appropriate qualities.

depressed states of mind.” Indeed, those who experienced the revelations “should worry about being ill in the way that insane, manic, or depressive people are” (p. 339).

In our own day, Kroll and Bachrach (2005) have pointed out that many extreme ascetics are likely to have had psychological problems as a result of self-inflicted sleep deprivation, self-harm (especially whipping oneself), and fasting (see also Kroll & Bachrach, 1982a, 1982b). Asceticism can produce psychological problems as well as result from them. This idea is not new, and the same concerns were voiced in the later Middle Ages. Gerson (1402/1998, p. 345) remarked that

excessive abstinence . . . brings incurable illness from brain damage and mental disorder. It happens then through mania or rage or other melancholy passions that phantasms become so deeply rooted and buried in the brain that they are thought to be true objects that appear outside the mind.

In some cases, this could proceed until the person believed he was something else, for example, a purple fish or dead. Henry of Langenstein (1388/1977, p. 70) claimed that austerity could produce sickness, and those who did not receive, or did not follow, the counsel of doctors could become incurable, sink into foolishness, and, at last, become quite mad. Gerson (1402/1998, p. 346) spoke of excessive fasting bringing on a “terrible trembling.” The anonymous author of *The Cloud of Unknowing* (Walsh, 1375/1981, p. 222) remarked of overenthusiastic novices that “the eyes of some of them are so set in their heads as though they were sheep suffering from brain disease and were near death’s door.” Although it is not easy to be sure, the frequency and phrasing of these comments suggest that they were based on observation as well as theory, and thus that some medieval would-be mystics did fall into states that led others to consider them mentally disordered.

Yet, in practice, excessive religious fervor did not often lead contemporaries to think those practicing it were mentally disordered. Kroll and Bachrach (1982b), reviewing the histories of 134 medieval visionaries (some consequently canonized), found only one who was regarded by onlookers as deranged. They identified four visionaries who might have been regarded as mentally disordered by late-20th-century standards, but none of them were so regarded in their own time.

An example of this is provided by Margery Kempe (1373–after 1436). Kempe was a late medieval Englishwoman who was clearly very religiously oriented but never quite managed to convince most of her contemporaries that her experiences were genuine. She wrote (or, more accurately, dictated) a book of her experiences that has been much studied by recent historians from a range of different perspectives (e.g., Atkinson, 1983; McEntire, 1992). One recent writer, Craun (2005), has suggested that, by present-day standards, she was mentally disordered.

According to Kempe’s (1436/1940, 1436/1985) report, she met with very mixed reactions from her contemporaries. Many of these reactions were to her habit of frequently breaking out with loud weeping, crying, and roaring, especially during religious events.⁷ Unsurprisingly, this behavior met with a good deal of disapproval from those who witnessed the outbursts, but she claimed that she was unable to control them. Many witnesses were hostile and regarded her as pretending, heretical, or under the influence of the devil, but other witnesses more sympathetically viewed her as genuinely visited by God, and indeed, enquiries by men in the religious hierarchy seem to have largely exonerated her. Interestingly, in her whole account, there is not a single example of hostile onlookers believing that she was mentally deranged (although some thought, during one of her outbursts, that she might have the “*fallyng euyt*” [epilepsy]; Kempe, 1436/1940, p. 105). Yet the author herself confessed to having gone out of her mind at an earlier stage

⁷ Her behavior was not original. Mary of Oignes was described as “shouting like a woman giving birth” during confession (McGinn, 1998, p. 34).

of her life before her religious experiences began (Kempe, 1436/1940, pp. 7–8), so the possibility of mental disorder was familiar to Kempe herself.

Kempe's autobiography gives a number of clues as to why her uncontrollable crying fits did not lead people to think she was mentally disordered. For example, she showed charity by caring for her husband as he aged (Kempe, 1436/1940, pp. 179–181) and was frequently concerned about the fate of the damned (e.g., pp. 144–146). An important element of the medieval view of mental disorder was that the incompetence of the individual was demonstrated by obvious failures of memory or reasoning. But generally, Kempe seems to have dealt with her world competently, if idiosyncratically. She was apparently able to detect would-be swindlers (pp. 55–58). According to her account, she successfully managed to complete long-distance travel from England to Rome, Santiago de Compostella, Danzig, and Jerusalem. She reported several times being questioned and even imprisoned by different ecclesiastical authorities, but each time, she was able to talk her way out of difficulties (e.g., pp. 32–38, 63–66, 121–128). In short, if we leave aside her religiously inspired crying fits and visions, she simply did not fit the contemporary model of mental disorder.

Another somewhat different example is given in a book written circa 1340 by an anonymous contemporary about Gertrude Rickeldey of Ortenberg (Mulder-Bakker, 2017). Following her widowhood in 1301, Rickeldey led a religiously centered life until she died in 1335, and the book is largely concerned with her spiritual experiences and how they gradually changed over time. The author was often at pains to stress Rickeldey's humility, willingness to take advice from others with regard to moderating her own self-discipline and austerity, frequent serenity and joy, conventional religious beliefs, charity, and aversion to interpersonal disputes.

During this time, an astute companion, Heilke of Staufenberg, seems to have helped to keep Rickeldey grounded. Indeed, the author remarked, "Had Gertrude not had Heilke, she would often have been mocked and she might even have lost her mind" (Mulder-Bakker, 2017, p. 131). A number of the incidents described in the book might justify this comment. Rickeldey bore four children, who all died young, yet when the last of these died, she did not cry for him (p. 127). Early on in her religious career, she could weep all one day and then laugh all through another (pp. 134–138). Occasionally, especially after attending a church service, she was apparently unable to move, and once unable to see either (pp. 135, 234). She was often described as moved by the spirit to ever greater austerity, and Heilke frequently had to persuade her to eat. Later in her life, when Rickeldey gave away all her property, she was suspected by her confessor of being misled by a devil, and Heilke seems to have briefly feared for her sanity (pp. 225–227). However, after the property issue had been resolved, the doubts, which were not directly related to a religious experience, were no longer mentioned.

Conclusions

Different societies and different eras think about mental disorder differently. As a recent example, since the creation of the first *Diagnostic and Statistical Manual of Mental Disorders* in 1952, the way in which mental disorders have been regarded in the United States has undergone considerable change. New disorders (e.g., hoarding) have appeared; others (e.g., homosexuality) have disappeared from the manual. Many disorders have been renamed and diagnostic criteria have been changed (e.g., Blashfield, Keeley, Flanagan, & Miles, 2014). Given this variation in one country over a relatively brief period of time, one might expect to find that late medieval concepts of mental disorder were different from ours, both in the way that mental disorders were characterized and in what was considered abnormal behavior. Medieval visions of a religious nature, asceticism, and even religiously oriented outbursts of the kind exhibited by Margery Kempe do not seem, in themselves, to have been regarded as symptomatic of mental disorder, although it was

widely recognized that mental disorders existed and that extreme asceticism could produce them.

The personal characteristics sought in those who might be credited with genuine religious experiences could be seen as desirable virtues for medieval people generally, at least from a religious viewpoint. Discretion, patience, calmness, and tranquility, and perhaps charity (in its broad sense) are present-day virtues, too, but humility, whose medieval meaning seems almost contrary to self-esteem, is not. Thus, medieval ideas of desirable personal characteristics also seem to have differed a little from our own.

Later medieval Europe possessed reasonably well-developed models of both mental disorders and how good, evil, and natural spirits might be responsible for unusual experiences. Knowledge of both of these models seems to have been common. In particular, knowledge of the models of mental disorder was not limited to physicians. Religious experiences were probably common in this era, and although there were often doubts about whether these were genuinely of divine, angelic, or saintly origin, those reporting them were not usually regarded as mentally disordered.

This article has shown how Huot's (2003) distinction between mad people and saints could actually be made in the later Middle Ages. The analysis shows that the behavior expected from the mentally disordered and the behavior expected from people who might have been believed to have genuine religious experiences were quite different. The behavioral difference was not restricted to, or even mostly based on, behavior observed when people actually had their religious experiences or shortly following them, but rather in behavior that could be seen over much longer time periods and much more varied settings, sometimes over a lifetime. Accounts of the lives of St. Francis of Assisi, Margery Kempe, and Gertrude Rickeldej do not portray their subjects as suffering from mental disorder, although in each case such a disorder is mentioned. Instead, the accounts stress the subjects' humility, charity, and other characteristics of people who, according to treatises on the discernment of spirits, might have had genuine religious experiences.

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Received September 3, 2018

Revision received November 13, 2018

Accepted December 6, 2018 ■

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